TEST TEST
MRN : **A1351656026**
Birthday : **2011-11-24**
Phone : **5555555555**
Visited on: 2024 Nov 22 13:30 (Age at visit: 13 years)
Electronically signed by: Teresa Asam-Jensen, MD on 2024-11-22 02:15 PM

CC

Thyroid

Subjective

Emily B - history of thyroid disorder (Hashimoto); as of the last visit wass placed on NP thyroid 100mg daily. She had tried levothyroxine when first dx after the birth of her last child -without much success. Numbers didn't seem to respond and didn't feel great.

Most recent labs:
1.2 TSH
T4 1.21 (direct)
T3 - WNL
TPO nearly 200
up from 130 (after going back to gluten diet)
cut it back out - went up instead - feel really tired; sleeps 2hrs more than spouse; unsure if its life or thyroid
+hair loss; skin normal; BMs normal; appetite about the same maybe more sensitive to alcohol and other rich/junk food
mood is good
working out: gaining stamina, feels good
sleeps through night 5d per week - kids wake up 1-2 days per week. Sleep: no snoring; light sleeper for kids but doesn't omove and seems sound. 8h
At dx cut out inflammatory foods - antibodies went down and felt better

No recent illnesses or unusual stressors; just a mild cold 2 weeks ago.
No change in supplements, no other medications.

ROS

12 pt ROS performed including Gen, HEENT, Resp, CV, GI, Skin - nothing additional to HPI

Medications

(Medication reconciliation last updated 11/22/2024 01:42 PM, TERESA ASAM-JENSEN - Performed)
NP Thyroid 120 mg tablet
Vitamin D3 125 mcg (5,000 unit) tablet

Allergies

No known allergies

Mental/Functional

Mood is good: denies depression, anxiety

Vitals

Height/Length	Weight	BMI	Blood Pressure	Temperature	Pulse	RR	SpO2
						17	

Recorded: 2024 Nov 22 14:08 by: Teresa Asam-Jensen

Objective

GEN: NAD, sitting comfortably facing screen, engaged easily in conversation.
HEENT: MMM to observation
NECK: supple, NT, FROM

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RESP: nonlabored breathing, no use of accessory of muscles of respiration
DERM: skin warm and dry
EXT: symmetric movement bilat/UE; no cyanosis/clubbing/edema
NEURO: AO x 3, grossly normal coordination and tone
PSYCH: judgment/insight intact, euthymic mood/ congruent affect; good eye contact

Labs Reviewed: Nov 2024
1.2 TSH
T4 1.21 (direct)
T3 - WNL
TPO nearly 200

Imaging: none
Cycle review: none

Assessment

Hashimoto thyroiditis (E06.3) Autoimmune thyroiditis modified 22 Nov, 2024

(Problem reconciliation performed by Teresa Asam-Jensen 02:10 PM 22 Nov 2024 Last updated)

Plan

Hashimoto thyroiditis

Clinically, not feeling great and c/o fatigue with unrestorative sleep. However, able to work out and gaining stamina and strength, supporting the lab results that indicate adequate thyroid function. History suggests against a sleep disorder, depression or hormonal dysfunction, but she may have chronic sleep deprivation that is not yet corrected based on the ages of her kids. Also may need combined T3 and T4 supplementation for best results. Discussed other ways to optimize immune regulation through diet or other medications (naltrexone) - resources sent.
Agreed to reduce NP3 to 50mg and add back levotyroxine 50mcg.
Check thyroid panel in 6 weeks - sent to LabCorps.
Call if feeling significantly worse and need to be seen earlier to reassess medications, otherwise schedule follow up for after labs are completed.